IN THE SUPREME COURT OF IOWA
No. 23-2025

ELLIE T. SHIMP, ZACH SHIMP and KERRI SHIMP,
Plaintiffs–Appellants,

v.

TIMOTHY A. GIBBONS, MD., CHAD H. BOYER, P.A.-C., JARED A. KNOWLES, P.A.-C, MASON CITY CLINIC, PC, MERCY HEALTH SERVICES-IOWA, CORP. d/b/a MERCYONE NORTH IOWA MEDICAL CENTER AND MERCY MEDICAL CENTER-NORTH IOWA,
Defendants–Appellees.

APPEAL FROM THE IOWA DISTRICT COURT
FOR CERRO GORDO COUNTY CASE NO. LACV072217
THE HONORABLE GREGG R. ROSENBLADT, PRESIDING

BRIEF OF AMICUS CURIAE IOWA HOSPITAL ASSOCIATION

Jeffrey R. Kappelman
AT0012719
Jacob T. Wassenaar
AT0015342

FINLEY LAW FIRM, PC
699 Walnut Street, Ste 1700
Des Moines, IA 50309
Telephone: 515-288-0145
Facsimile: 515-288-2724
E-mail: jkappelman@finleylaw.com
jwassenaar@finleylaw.com

ATTORNEYS FOR AMICUS CURIAE IOWA HOSPITAL ASSOCIATION

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IDENTITY AND INTEREST OF AMICUS CURIAE

The Iowa Hospital Association is a nonprofit organization that advocates for and supports Iowa's hospitals and health systems throughout the state. The Iowa Hospital Association's purpose is to reimagine health care in Iowa for life-changing outcomes. In fulfilling this purpose, one of the Iowa Hospital Association's strategic priorities is centered on improving Iowans' access to health care by collaborating with its member hospitals to innovate and promote hospital workforce recruitment and retention strategies.[1]

The present case raises substantial issues involving the liability of hospitals, health systems, and healthcare providers in treating personal injuries. Medical care almost invariably follows personal injuries, so providers of medical care are disproportionately affected by enhanced-injury principles of comparative fault. As an amicus curiae, the Iowa Hospital Association offers the Court additional perspective on the ramifications of its decision in this case on hospitals and health systems throughout the state. Accordingly, the Iowa Hospital Association has a substantial interest in this action and its outcome, as do the healthcare providers throughout Iowa who are supported by the Association.

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[1]For example, the Iowa Hospital Association has published literature outlining the importance of hospitals and healthcare systems in Iowa and the challenges they face. See Hosp. Ass'n, *Mission Essential* (2026), https://ihaonline.org/wp-content/uploads/2026/03/26Mission-Essential-2.pdf. Among the challenges include thousands of open positions in Iowa's hospitals. See *id.* at 4.

STATEMENT OF AUTHORSHIP AND FUNDING

The Iowa Hospital Association is represented by the undersigned counsel of the Finley Law Firm, PC. Counsel for the Association authored this brief in full. No party, party's counsel, or other person or entity contributed money to fund the preparation or submission of this briefing.

ARGUMENT

Hospitals, health systems, and healthcare providers play a role in nearly every personal-injury case as the entities called upon to treat, repair, and/or mitigate, to the extent possible, injuries suffered by personal-injury plaintiffs. In negligence actions involving subsequent medical care, the plaintiff's theory begins with the same premise: but for the original tortfeasor's conduct, the plaintiff would never have needed medical treatment at all. Medical providers are obligated—by law, ethics, and professional duty—to treat injuries as they are. Modern tort litigation therefore places hospitals and medical providers in a uniquely exposed position when those injuries are not resolved or later worsen. Hospitals and medical providers are routinely drawn into litigation not because they caused the underlying injury but because they treated it. And the injuries presented are often catastrophic to begin with.

Instead of exposing Iowa's medical providers to liability for the entirety of an injured party's claimed damages, Iowa's comparative-fault regime protects

against that inequity by limiting defendants' liability to the harm they actually caused. The Iowa Comparative Fault Act requires allocation of fault among all actors whose conduct contributed to the plaintiff's ultimate injury, including original tortfeasors and subsequent medical providers.

Here, however, the Shimps effectively ask this Court to impose liability on medical providers for the entirety of an injury, including harm caused by the original tortfeasor whose conduct necessitated the medical treatment in the first place, by asking this Court to prevent the allocation of fault to the original tortfeasor and force medical providers to bear the totality of a plaintiff's enhanced injuries on the verdict form. That approach is inconsistent with both the text and purpose of Iowa's comparative-fault framework and exposes medical providers to liability for the totality of an injury they did not fully cause.

The Iowa Hospital Association therefore submits this amicus curiae brief to aid the Court in preserving the fundamental principle that defendants should be liable only for their respective share of fault and to ensure that medical providers are not disproportionately punished for rendering care to injured Iowans. The Iowa Hospital Association respectfully urges this Court affirm the district court and allow the medical providers to compare fault with the original tortfeasors.

I. Iowa's Comparative Fault Act requires fault to be allocated among all actors who contributed to the plaintiff's ultimate injury.

Our legislature established comparative fault "as the basis for liability in relation to claims for damages arising from injury to or death of a person." See 1984 Iowa Acts, ch. 1293. Housed in Iowa Code chapter 668, the Iowa Comparative Fault Act provides broad comparative-fault principles:

> In the trial of a claim involving the fault of more than one party to the claim, including third-party defendants and persons who have been released pursuant to section 668.7, the court . . . shall instruct the jury to answer special interrogatories or, if there is no jury, shall make findings, indicating all of the following: . . . The percentage of the total fault allocated to each claimant, defendant, third-party defendant, person who has been released from liability under section 668.7, and injured or deceased person whose injury or death provides a basis for a claim to recover damages for loss of consortium, services, companionship, or society. For this purpose the court may determine that two or more persons are to be treated as a single party.
>
> Iowa Code § 668.3(2).

The aim of chapter 668 is proportional compensation. See *Godbersen v. Miller*, 439 N.W.2d 206, 208 (Iowa 1989). While section 668.3(1) prevents a plaintiff from being compensated for fault that he or she should fairly bear, section 668.3(2) exists to make defendants pay in proportion to their fault. See *Godbersen*, 439 N.W.2d at 208; *Kopsas by Kopsas v. Iowa Great Lakes Sanitary Dist. of Dickinson Cnty.*, 407 N.W.2d 339, 341 (Iowa 1987). Our legislature therefore created a comparative-fault regime that holds each actor accountable in proportion to their responsibility.

"Fault" is defined broadly by the Act and covers all acts or omissions that are in any measure negligent. See Iowa Code § 668.1. The Act is not limited temporally or contextually. And it explicitly includes released parties in its analysis. See Iowa Code § 668.3(2); see also Iowa Code § 668.7 (stating "the claim of the releasing person against other persons is reduced by the amount of the released person's equitable share of the obligation . . . ."). Given the breadth of the Act, this Court has declined to read in exceptions. See *Jahn v. Hyundai Motor Co.*, 773 N.W.2d 550, 560 (Iowa 2009) ("While an exception to the application of comparative fault principles for enhanced injury cases might be supported on policy grounds, the legislature has not provided for such an exception."). Thus, comparative fault applies even when the actor's negligence occurs at different times and in different contexts.

Yet the Act is not exclusively focused on the comparison of negligent conduct but instead also requires the comparison of the degree to which each defendant's negligence caused the injuries claimed. See Iowa Code § 668.1(2) ("The legal requirements of cause in fact and proximate cause apply both to fault as the basis for liability and to contributory fault."); Iowa Code § 668.3(3) ("In determining the percentages of fault, the trier of fact shall consider both the nature of the conduct of each party and the extent of the causal relation between the conduct and the damages claimed." (emphasis added)). The Act requires liability to

be apportioned according to the relevant fault of all actors who contributed to plaintiff's ultimate injury. See, *e.g.*, Iowa Civ. Jury Instr. 300.6.

With this in mind, the Court of Appeals' analysis of the propriety of fault allocation in this case is revealed as too narrowly focused on the question of negligence alone. See *Shimp v. Gibbons*, 29 N.W.3d 641 (Table), No. 23-2025, 2025 WL 3165936, at *2–5 (Iowa Ct. App. 2025). The Court of Appeals' opinion suggests that the question of comparison of fault only encompasses the issue of negligence. See *id.* at *3 ("Applying that reasoning here, only if the medical providers were at fault for misdiagnosing [the injured party]'s artery occlusion, would the extent of her leg injuries become potentially relevant to causation or scope of liability questions. And on those questions, whether her injuries resulted from her own negligence or the negligence of the jet ski defendants would make no difference." (emphasis in original)). The Court of Appeals based its analysis on this Court's opinion in *DeMoss v. Hamilton*, 644 N.W.2d 302 (Iowa 2002). But the *DeMoss* Court also construed comparative fault too narrowly by similarly concluding that its focus was limited to the question of negligence. See *DeMoss*, 644 N.W.2d at 307 ("Any 'fault' on [the plaintiff]'s part for conduct contributing to his heart attack is simply irrelevant to the question of medical negligence underlying [the plaintiff]'s cause of action. Only if [the defendant] were found at fault for alleged misdiagnosis or treatment would [the plaintiff]'s state of health

become potentially relevant to the remaining questions of proximate cause, lost chance of survival and life expectancy." (emphasis in original)).

Contrary to these analyses, the focus of the Act is not simply "fault" but more broadly "liability." See Iowa Code ch. 668 ("Liability in Tort—Comparative Fault.") The plain language of the Act explicitly requires the consideration of causation as well as negligence in the liability analysis. See Iowa Code §§ 668.1(2), 668.3(3); see also *Jahn v. Hyundai Motor Co.*, 773 N.W.2d 550, 559 (Iowa 2009) ("In this language [of Iowa Code section 668.3(3)], we conclude that in enhanced injury cases, the legislature has directed that the casual [sic] relation between the conduct of a [a party] and the resulting damages is one of the two elements to be considered in assigning a percentage of liability, but is not solely determinative as to the allocation of fault."). Thus, where the negligent conduct of one party combines with the negligence of another to cause injury, each party is entitled to a comparison of fault in the determination of its ultimate liability.

II. Original tortfeasors and subsequently negligent medical providers contribute to a distinct chain of harm which requires the comparison of fault.

At its core, Iowa law holds tortfeasors responsible for the natural and foreseeable consequences of their actions. See *Jahn v. Hyundai Motor Co.*, 773 N.W.2d 550, 559–60 (Iowa 2009). And medical treatment is among the most foreseeable consequences of physical injury. An original tortfeasor is liable for

subsequent medical care and expenses related to that care even when that tortfeasor has no control over the services provided or the ultimate costs of the services to the patient. See *id.*

Iowa courts further recognize that even negligence in providing subsequent medical services can be a natural and foreseeable consequence of tortious conduct for which the original tortfeasor is responsible. See *Jahn*, 773 N.W.2d at 559; *Casey v. Koos*, 323 N.W.2d 193, 197 (Iowa 1982); *Smith v. Conn*, 163 N.W.2d 407, 410 (Iowa 1968). These principles hold tortfeasors accountable for the downstream results of their actions, including and especially subsequent medical care. See *Casey*, 163 N.W.2d at 197. Moreover, these principles recognize that the ultimate injuries are, at least in part, caused by the original tortious conduct, even in the setting of subsequent negligent medical care.

Considering these foundational principles of Iowa law, excluding original tortfeasors and/or released parties from fault allocation is inconsistent with the Act's text and purpose. Circumstances involving subsequent medical care and enhanced injuries are precisely why comparative fault exists. The plaintiff alleges that multiple actors contributed to a single injury subsequently enhanced. Comparative fault allows the jury to determine the relative responsibility of each actor and to allocate liability accordingly. This is true even if the original tortfeasor would ultimately be liable for all the foreseeable harm. Nothing in Iowa's

comparative-fault framework suggests that responsibility should be assigned solely to the last alleged wrongdoer in the causal chain, which is almost invariably medical providers.

Importantly, these principles do not work in reverse: Iowa law does not hold medical providers responsible for acts that took place before the care at issue. This is counter to a foreseeability analysis. Comparative-fault principles based on foreseeability of harm were never intended to relieve original tortfeasors of responsibility in a comparative-fault system or to impose disproportionate liability on medical providers who merely responded to an injury created by another. Precluding the comparison of fault would effectively end the original tortfeasor's proportional responsibility at the hospital door, even though the need for medical treatment arose directly from that tortfeasor's conduct and the ultimate injuries were caused, in part, by the original tortfeasor's conduct. In situations where there are multiple alleged tortfeasors, those at the end of the causal chain are left with the totality of a plaintiff's claim of damages for enhanced injuries, while those in the beginning and in the middle can escape proportional liability altogether.

While the Court of Appeals recognized that medical providers should not be liable for injuries to the extent they were caused, in whole or in part, by the acts or omissions of another, it limited this inquiry to one of causation. See *Shimp v. Gibbons*, 29 N.W.3d 641 (Table), No. 23-2025, 2025 WL 3165936, at *3 (Iowa Ct.

App. 2025). But as explained above, the issue of causation is not distinct from the comparative-fault analysis but is instead an explicit and necessary aspect of determining liability in a comparative-fault scheme. See Iowa Code §§ 668.1(2), 668.3(3). Because both the original tortfeasor and the medical provider allegedly contributed to the plaintiff's ultimate injury, both must be included in the fault-allocation process.

III. Iowa's caselaw does not exclude medical providers from the protection of Iowa's Comparative Fault Act or proportional liability.

While the Shimps argue that caselaw from this Court, namely *DeMoss v. Hamilton*, 644 N.W.2d 302 (Iowa 2002), precludes comparative fault, this approach ignores relevant distinctions made in the caselaw involving the comparative fault of medical providers.

Iowa caselaw already draws a distinction between injuries associated with the general health of a patient and specific injuries associated with a tortfeasor. In *DeMoss*, for example, this Court ruled that medical providers could not compare fault with patients when the issue concerned a plaintiff's "state of health" when presenting to medical professionals. See *DeMoss v. Hamilton*, 644 N.W.2d 302, 305–07 (Iowa 2002). However, the *DeMoss* Court simultaneously recognized that a comparative-fault defense could apply if the plaintiff failed to follow the doctor's instructions as to follow-up care. See *DeMoss*, 644 N.W.2d at 306 ("[A] patient's

failure to cooperate with or follow reasonable directions for the treatment of an ailment may be relevant in the appropriate case" as evidence of comparative fault.). Meanwhile, in *Jahn*, this Court ruled that comparative-fault principles apply in enhanced-injury cases where a separate tortfeasor was part of the causal chain of an injury. See *Jahn v. Hyundai Motor Co.*, 773 N.W.2d 550, 559–61 (Iowa 2009). The *Jahn* Court held tortfeasors responsible for the natural and foreseeable consequences of specific acts and refused to exclude enhanced-injury cases from Iowa's comparative-fault regime. See *id.* Taken together, *DeMoss* and *Jahn* instruct that comparative fault applies when actors can be held responsible for specific negligent acts that cause injury.

Other cases echo this principle. In *Mulhern*, this Court construed chapter 668 to permit a comparative-fault defense in a medical-negligence action arising from a noncustodial suicide. See *Mulhern v. Cath. Health Initiatives*, 799 N.W.2d 104, 117 (Iowa 2011). Section 668.3 exists to make defendants pay in proportion to their fault, said this Court, so when the estate sought to fully recover from medical providers for the patient's later self-harm without any reduction for her own responsibility in that harm, this Court rejected that attempt. See *id.* at 112–17 (citing *Godbersen v. Miller*, 439 N.W.2d 206, 208 (Iowa 1989)). Notably, the Court held that the act of suicide can be construed as "negligent" within the meaning of section 668.1(1). *Id.* at 114.

Similarly, in *Ichelson*, this Court allowed the introduction of evidence regarding subsequent medical procedures "to allow the jury to sort out what effect, if any, those procedures had on Ichelson's condition" because there was evidence that those procedures could have caused a problem in question. See *Ichelson v. Wolfe Clinic, P.C.*, 576 N.W.2d 308, 310–11 (Iowa 1998). Thus, Iowa law has long trusted Iowa juries to parse out the respective fault of the parties in question to determine their respective liability when specific negligence and causal relation can be identified. See *Ichelson*, 576 N.W.2d at 310–11; *Mulhern*, 799 N.W.2d at 112–17.

Iowa caselaw thus draws a distinction between injuries associated with the general health and wellness of a patient, which fall outside the bounds of comparative fault (like in *DeMoss*), and specific injuries associated with a tortfeasor, which fall within the bounds of comparative fault (like in *Ichelson*, *Jahn*, and *Mulhern*). The *Ichelson–DeMoss–Jahn–Mulhern* line of cases stand for the proposition that a patient's general health and wellness when presenting is too remote a cause as to affect the proportional liability of treating medical providers, but when a specific action can be causally linked to the care, comparative fault must apply. See also *Lee v. Small*, 829 F. Supp. 2d 728, 750 n.8 (N.D. Iowa 2011) ("[I]t appears likely that the comparative fault concepts of the Comparative Fault Act would also apply to the issue of whether or not a physician's or hospital's

malpractice or negligence increased a plaintiff's injuries from conduct of an original tortfeasor.").

In situations where an original tortfeasor caused a plaintiff to seek subsequent medical care, like here, the patient's general health and wellness accumulated over time is not at issue. Instead, a specific, identifiable injury is presented. When plaintiffs seek medical care for a specific and identifiable injury caused by a tortfeasor, the original tortfeasor (and possibly the plaintiff herself) had at least some agency over the injury such that the jury is entitled to compare their fault. The breadth of Iowa's Comparative Fault Act requires the application of comparative-fault principles in such a situation, and Iowa caselaw supports such a result.

IV. Vicarious liability for original tortfeasors does not except them from comparison of fault.

This Court has likened an original tortfeasor's exposure for the downstream effects of its torts to vicarious liability for the subsequent harm. See *State ex rel. Miller v. Philip Morris Inc.*, 577 N.W.2d 401, 406 (Iowa 1998). And the Act allows courts to treat two or more persons "as a single party" for purposes of fault allocation. See Iowa Code § 668.3(2)(b). The Court of Appeals, relying heavily on vicarious-liability principles, characterized the original-tortfeasor doctrine as a form of vicarious liability and concluded there was "no fault to be compared." See

Shimp v. Gibbons, 29 N.W.3d 641 (Table), No. 23-2025, 2025 WL 3165936, at *4 (Iowa Ct. App. 2025).

But the Court of Appeals carried the analogy of the original-tortfeasor rule as a form of vicarious liability too far, improperly inverting the foreseeability analysis and collapsing necessary distinctions between successive tortfeasors. This Court recently explained that comparative-fault principles are left undisturbed between employees and employers who have admitted vicarious liability because they are treated as a "single unit" with "coextensive" liability such that they are treated as a single party for purposes of the Comparative Fault Act. See *Baldwin v. Cent. Iowa Hosp. Corp.*, __ N.W.3d __, No. 24-1645, 2026 WL 1441577, at *7 (Iowa 2026) (citing Iowa Code § 668.3(2)).

But original tortfeasors and subsequent medical providers are not a "single unit" and do not have "coextensive" liability such that the Act treats them as a single party. They do not act in concert, have not breached a common duty to the plaintiff, and do not have a relationship triggering the application of vicarious liability. See *Banks v. Elks Club Pride of Tennessee 1102*, 301 S.W.3d 214, 224 (Tenn. 2010) (explaining the same). They are instead two separate sets of actors.

An original tortfeasor does not cease to be a separate causal actor merely because another party later became involved. While the original tortfeasor is held liable for all foreseeable harm that follows, the harm that follows is still separate in

kind, and separate tortfeasors cannot be collapsed into a single party for purpose of avoiding the comparison of fault—despite a label of "vicarious liability." Accordingly, original tortfeasors and subsequent medical providers cannot be "a single party" for purposes of fault allocation. See Iowa Code § 668.3(2). Allocation of fault still applies, and the statement by the Court of Appeals that "there is no fault to be compared" because of the mere label of vicarious liability must be rejected. See *Shimp v. Gibbons*, 29 N.W.3d 641 (Table), No. 23-2025, 2025 WL 3165936, at *4 (Iowa Ct. App. 2025).

The Restatement provides a reasonable approach which accords with the above authorities while maintaining comparative-fault principles. See Restatement (Third) of Torts: Phys. & Emot. Harm § 35 (2010). The Restatement is aligned with the original-tortfeasors principles noted above:

> An actor whose tortious conduct is a factual cause of harm to another is subject to liability for any enhanced harm the other suffers due to the efforts of third persons to render aid reasonably required by the other's injury, so long as the enhanced harm arises from a risk that inheres in the effort to render aid.

See Restatement (Third) of Torts: Phys. & Emot. Harm § 35 (2010).[2] Of particular relevance here, the Restatement requires that when two or more defendants are liable for enhanced harm suffered by a plaintiff, "each of the

[2]This amicus curiae acknowledges that the litigants disagree as to whether the damages claimed are divisible. But Iowa Code chapter 668 makes no such distinction. Comparative-fault principles apply to divisible and indivisible injuries alike, subject to limitations concerning the amount of any divisible injuries. See *Jahn v. Hyundai Motor Co.*, 773 N.W.2d 550, 560 (Iowa 2009).

defendants is held liable for the amount of damages reflecting the enhanced harm discounted by the comparative share of responsibility assigned by the factfinder to that defendant." See *id.* cmt. d; see also *id.* illus. 4 (illustrating comparative-fault principles). In other words, the Restatement retains the original-tortfeasor doctrine while simultaneously upholding the Iowa Comparative Fault Act's requirement of proportional compensation, holding each actor accountable in proportion to their responsibility.

This amicus curiae urges the Court to join other states in harmonizing their respective comparative-fault requirements with principles from the Restatement to allow the comparison of fault between all allegedly negligent parties when enhanced harm is alleged. See, *e.g.*, *Cramer v. Starr*, 375 P.3d 69, 75–76 (Ariz. 2016); *Banks v. Elks Club Pride of Tennessee 1102*, 301 S.W.3d 214, 220–28 (Tenn. 2010). Any contrary rule would disproportionately affect medical providers and would stand opposed to well-worn principles of comparative fault.

V. Public policy strongly favors permitting comparison of fault between original tortfeasors and subsequent medical providers

Comparative fault reflects the commonsense principle that liability should correspond to responsibility. See *Godbersen v. Miller*, 439 N.W.2d 206, 208 (Iowa 1989). If an original tortfeasor causes a serious injury and a medical provider later contributes to an enhanced injury, fairness requires that both actors be included in

the fault-allocation process. Otherwise, medical providers may be held fully responsible for damages attributable, at least in part, to the conduct of others.

Such a rule would create strong incentives for defensive medicine and could discourage providers from undertaking higher-risk elective care. This Court previously noted that eliminating a comparative-fault defense in medical settings involving noncustodial suicide cases "would make bad public policy" by needlessly affecting how medical services are provided to suicidal patients. See *Mulhern v. Cath. Health Initiatives*, 799 N.W.2d 104, 121–22 (Iowa 2011) (citing *Hobart v. Shin*, 705 N.E.2d 907, 911 (Ill. 1998)). Eliminating such a defense in enhanced-injury cases presents similar risks because, in elective situations involving injuries caused by others, medical providers may openly decline to pursue elective procedures or would be encouraged to practice overly defensive medicine. Either approach artificially alters the plan of care provided to patients, reduces the availability of healthcare to Iowans, and needlessly increases the cost of medical care and the time and efforts of medical personnel. See *Mulhern*, 799 N.W.2d at 121–22 (noting similar rationale).[3]

Still other medical providers are required by law to treat patients regardless of the risks. The federal Emergency Medical Treatment and Active Labor Act

[3]The Iowa Hospital Association estimates thousands of open positions in Iowa hospitals already. See Iowa Hosp. Ass'n, *Mission Essential* 4 (2026), https://ihaonline.org/wp-content/uploads/2026/03/26Mission-Essential-2.pdf.

("EMTALA"), 42 U.S.C. § 1395dd, requires hospitals that receive Medicare funding to treat and stabilize all patients with emergency medical conditions. Providers of emergency care are distinctly at risk when patients present with catastrophic injuries because they are required by EMTALA to provide necessary stabilizing treatment at the hospital. See 42 U.S.C. § 1395dd(b)(1)(A). Providers of emergency medicine would thus be forced to bear the burden of increased liability without the protection of comparative fault. Regardless of whether the care is elective or mandatory, medical-malpractice insurers will surely notice the change in Iowa's comparative-fault principles, and premiums will rise accordingly.

Moreover, a rule precluding comparison of fault would produce arbitrary results, open the door to double recovery, and encourage strategic litigation choices. Absent application of comparative fault, plaintiffs can be made doubly whole by pursuing the totality of their injuries against two sets of tortfeasors respectively.

Upholding comparative-fault principles, on the other hand, would avoid these outcomes, streamline litigation, and protect medical providers from disproportional liability for harm they did not cause. This would presumably have a positive impact on the cost and availability of medical services for Iowans, which this Court has repeatedly referenced as an important objective. See, *e.g.*, *Struck v. Mercy Health Servs.-Iowa Corp.*, 973 N.W.2d 533, 541 (Iowa 2022); *Hantsbarger*

v. Coffin, 501 N.W.2d 501, 504 (Iowa 1993) (en banc); *Thomas v. Fellows*, 456 N.W.2d 170, 173 (Iowa 1990). Public policy therefore encourages—and the Iowa Comparative Fault Act requires—that comparative-fault principles apply to enhanced-injury cases such as this.

CONCLUSION

For these reasons, the Iowa Hospital Association respectfully urges this Court to hold that Iowa's comparative-fault framework permits the comparison of fault between original tortfeasors and subsequent medical providers whose alleged negligence contributed to the plaintiff's ultimate injury. The factfinder should be permitted to allocate responsibility among all actors whose conduct allegedly contributed to the plaintiff's injuries. Any contrary rule would improperly sever liability from relative responsibility, expose medical providers to disproportionate liability for injuries they did not create, and undermine the central purpose of Iowa's Comparative Fault Act. The Court should therefore affirm the district court, reverse the decision of the Court of Appeals, and uphold the principle that each defendant is liable only for the harm attributable to its own fault.

Respectfully Submitted,

/s/ Jeffrey R. Kappelman
Jeffrey R. Kappelman AT0012719
/s/ Jacob T. Wassenaar
Jacob T. Wassenaar AT0015342
FINLEY LAW FIRM, PC
699 Walnut Street, Ste 1700
Des Moines, IA 50309
Telephone: 515-288-0145
Facsimile: 515-288-2724
E-mail: jkappelman@finleylaw.com
jwassenaar@finleylaw.com
ATTORNEYS FOR AMICUS CURIAE
IOWA HOSPITAL ASSOCIATION

CERTIFICATE OF COMPLIANCE

This brief complies with the typeface requirements and type-volume limitations of Iowa Rs. App. P. 6.903(1)(g), 6.903(1)(i)(1), and 6.906(4) because this brief has been prepared in a proportionally spaced typeface using Century Schoolbook in size 14 font and contains 4,294 words, excluding the parts of the brief exempted by Iowa R. App. P. 6.903(1)(i)(1).

/s/ Jacob T. Wassenaar DATE: June 5, 2026

CERTIFICATE OF SERVICE AND FILING

I hereby certify that on June 5, 2026, the undersigned or someone acting on his behalf did serve this Brief of Amicus Curiae Iowa Hospital Association with the Iowa Supreme Court and upon counsel for the other parties appearing in this appeal through the EDMS system to:

Ryan G. Koopmans

KOOPMANS LAW GROUP LLC

500 East Court Ave., Suite 420

Des Moines, IA 50309

Phone: 515-978-1140

Email: ryan@koopmansgroup.com

H. Daniel Holm, Jr.

Eashaan Vajpeyi

BALL, KIRK & HOLM, P.C.

3324 Kimball Avenue

P.O. Box 2696

Waterloo, IA 50704

Phone: 319-234-2638

Email: evajpeyi@ballkirkholm.com

bkh@ballkirkholm.com

ATTORNEY FOR PLAINTIFFS–APPELLANTS

Frederick T. Harris

LAMSON, DUGGAN & MURRAY

1045 76th St., Ste. 3000

West Des Moines, IA 50266

Phone: 515-513-5003

ATTORNEY FOR DEFENDANTS-APPELLEES TIMOTHY A. GIBBONS, MD; CHAD H. BOYER, PA-C; JARED A. KNOWLES, PA-C; and MASON CITY CLINIC

Desirée Kilburg

Paul J. Esker

BRADLEY & RILEY, PC

404 E. College St., Ste. 400

Iowa City, IA 52240

Phone: 319-466-1511

ATTORNEYS FOR DEFENDANTS-APPELLEES MERCY HEALTH SERVICES-IOWA CORP. d/b/a MERCYONE NORTH IOWA MEDICAL CENTER and MERCY MEDICAL CENTER – NORTH IOWA

/s/ Jacob T. Wassenaar

Jeffrey R. Kappelman AT0012719

Jacob T. Wassenaar AT0015342

FINLEY LAW FIRM, PC

699 Walnut Street, Ste 1700

Des Moines, IA 50309

Telephone: 515-288-0145

Facsimile: 515-288-2724

E-mail: jkappelman@finleylaw.com

jwassenaar@finleylaw.com

ATTORNEYS FOR AMICUS CURIAE IOWA HOSPITAL ASSOCIATION